KEY BENEFIT ADMINISTRATORS

OPPORTUNITY REVIEW

DIRECT-TO-EMPLOYER GTM STRATEGY

Exploring a Wooster Community Hospital DTE Health Plan

KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
  - 3,000+ Corporate Clients
  - 3 Million Members under Management
  - 700 Employees

KEY BENEFIT
ADMINISTRATORS

Our Founding and Heritage:

46 Years of Striving for Honesty in Healthcare

Founded Key Benefit Administrators in 1979

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator focused on honest healthcare delivery.

Sold to Blue Cross/Blue Shield in 1985

Strategic partnership with BCBS of Indiana (then part of the Associated Group) provided capital and market access while maintaining operational independence.

Reacquired KBA in 1992

Larry bought the business back from BCBS in 1992, returning it to complete independence and patient-focused mission alignment.

Built the Largest Independent TPA in the U.S.

Under his leadership since the buyback, KBA has become the country's largest independently owned third-party administrator, managing millions of members across diverse markets.

Never Forgotten Who Gives Healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

MARKET LEADER IN PRIVATE LABELED DTE PLANS

KEY BENEFIT ADMINISTRATORS

Wooster Community Hospital
HEALTH SYSTEM

DIRECT-TO-EMPLOYER (DTE)

***Your* Health Plan Focused on a Direct Relationship with Self-Funded Employers in *Your* Market**

- ✓ **YOU** Pick the Name of the Health Plan
- ✓ **YOU** Determine the Fees Charged
- ✓ **YOU** Practice the Medicine
- ✓ **YOU** Receive the Profits
- ✓ **WE** Make it Successful

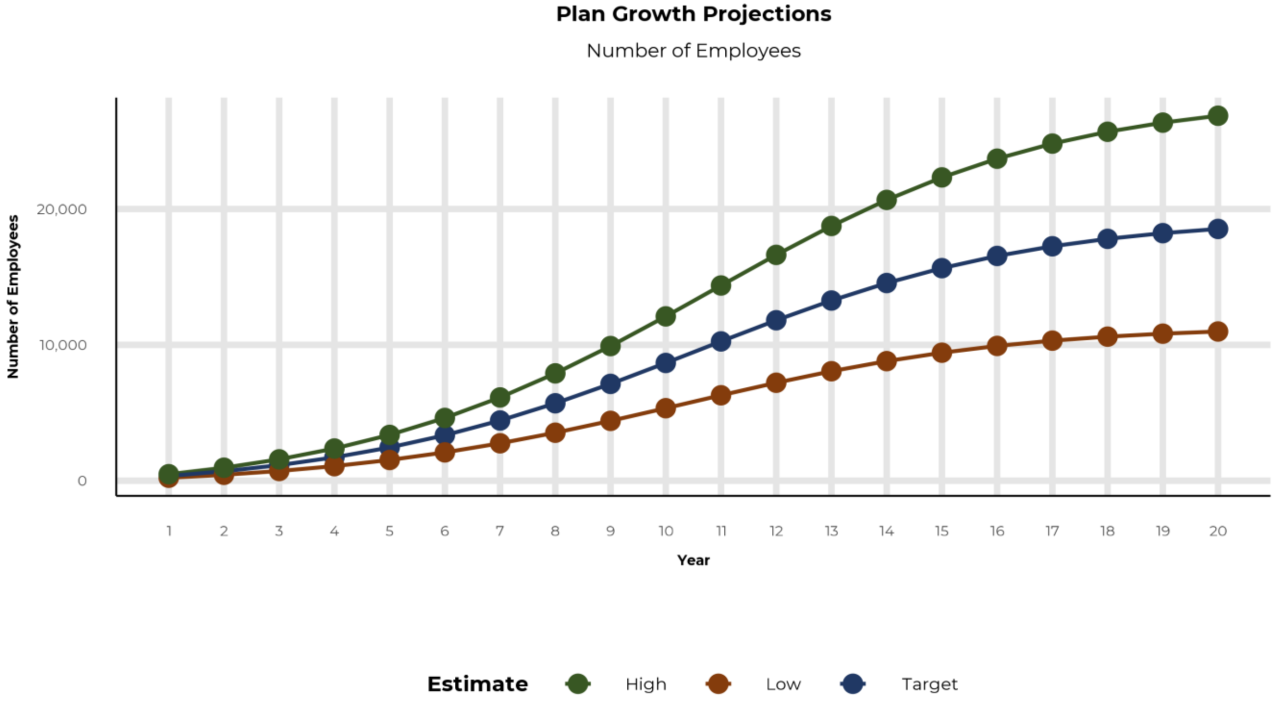
PLAN POTENTIAL GROWTH PATHS

Self Funded 127K

Insured 182K

Working Age 255K

Population 447K



Estimates includes the Wooster Community Hospital service area in Ashland, Holmes, Medina, Stark, Summit, and Wayne Counties

GROWING POWERFUL, LASTING PROFITABILITY

Early Stage
Gaining Name Recognition, Early Growth

New Revenue: **$10.21M**

New Profit: **$0.73M**

Growth Stage
Strong Reputation, Strong Growth

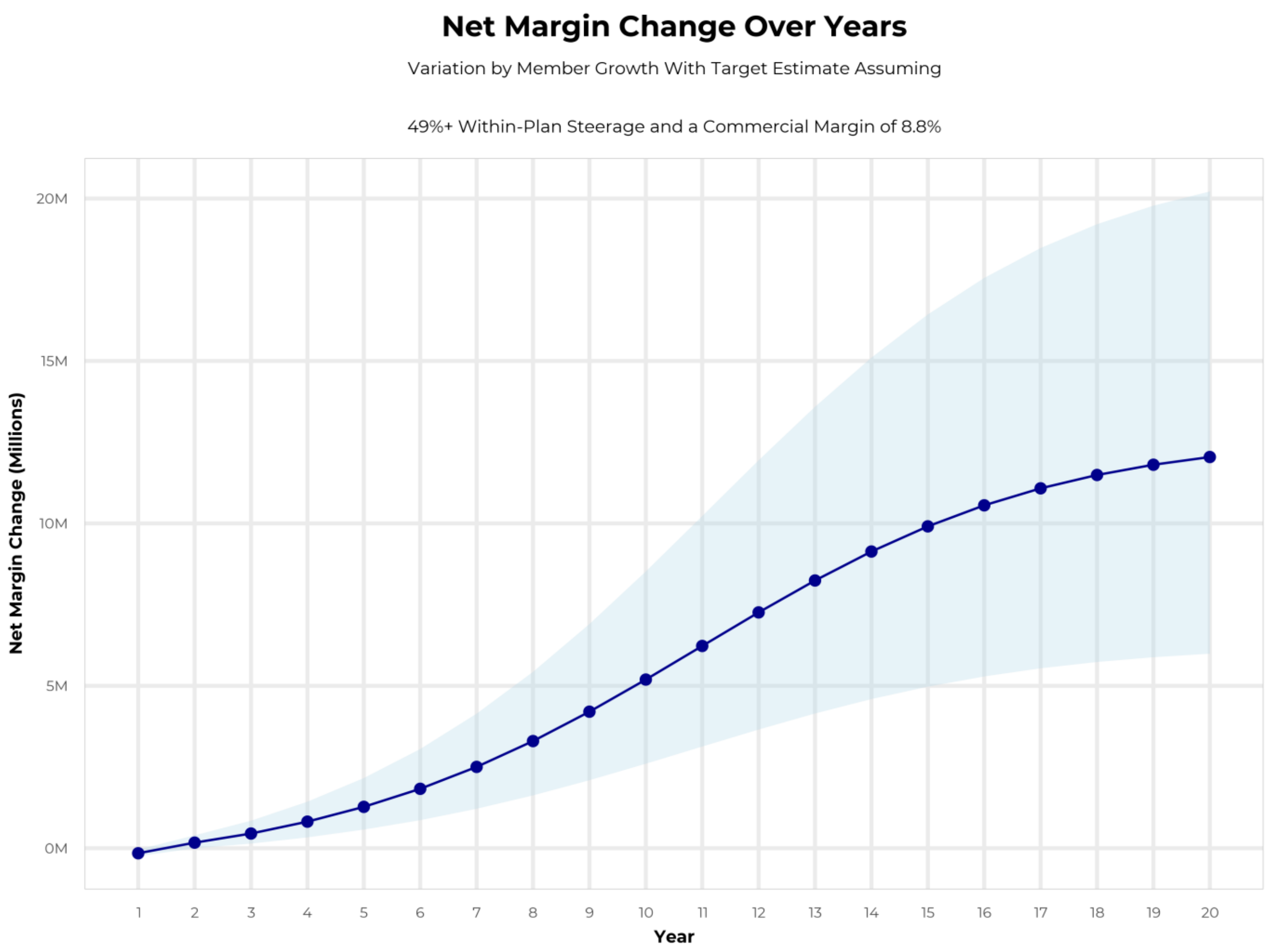
New Revenue: **$59.61M**

New Profit: **$5.28M**

Mature Stage
High Profitability, Market Saturation

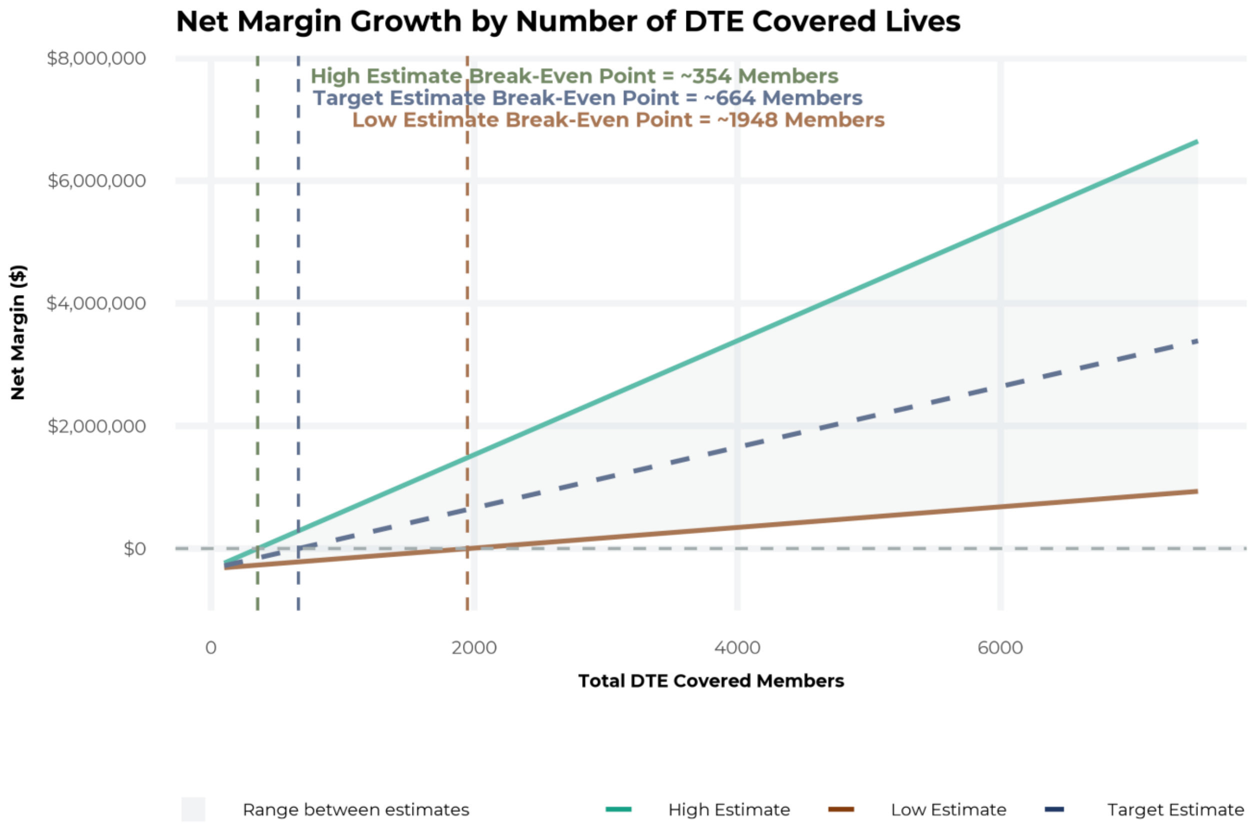
New Revenue: **$120.84M**

New Profit: **$10.86M**



KEY BENEFIT ADMINISTRATORS

MODEL ASSUMPTIONS AND BREAK EVEN



Patient Steerage Assumptions

KBA currently estimates that Wooster Community Hospital has 2% inpatient market share, and 2% outpatient market compared to hospitals/clinics in the Wooster Community Hospital service area (see slide 8). The DTE tiered structure will create immediate steerage, with additional steerage over time from patient bonus benefits and the patient convenience card (see slide 34 for estimates of target steerage).

Low	High
10% below target	10% above target

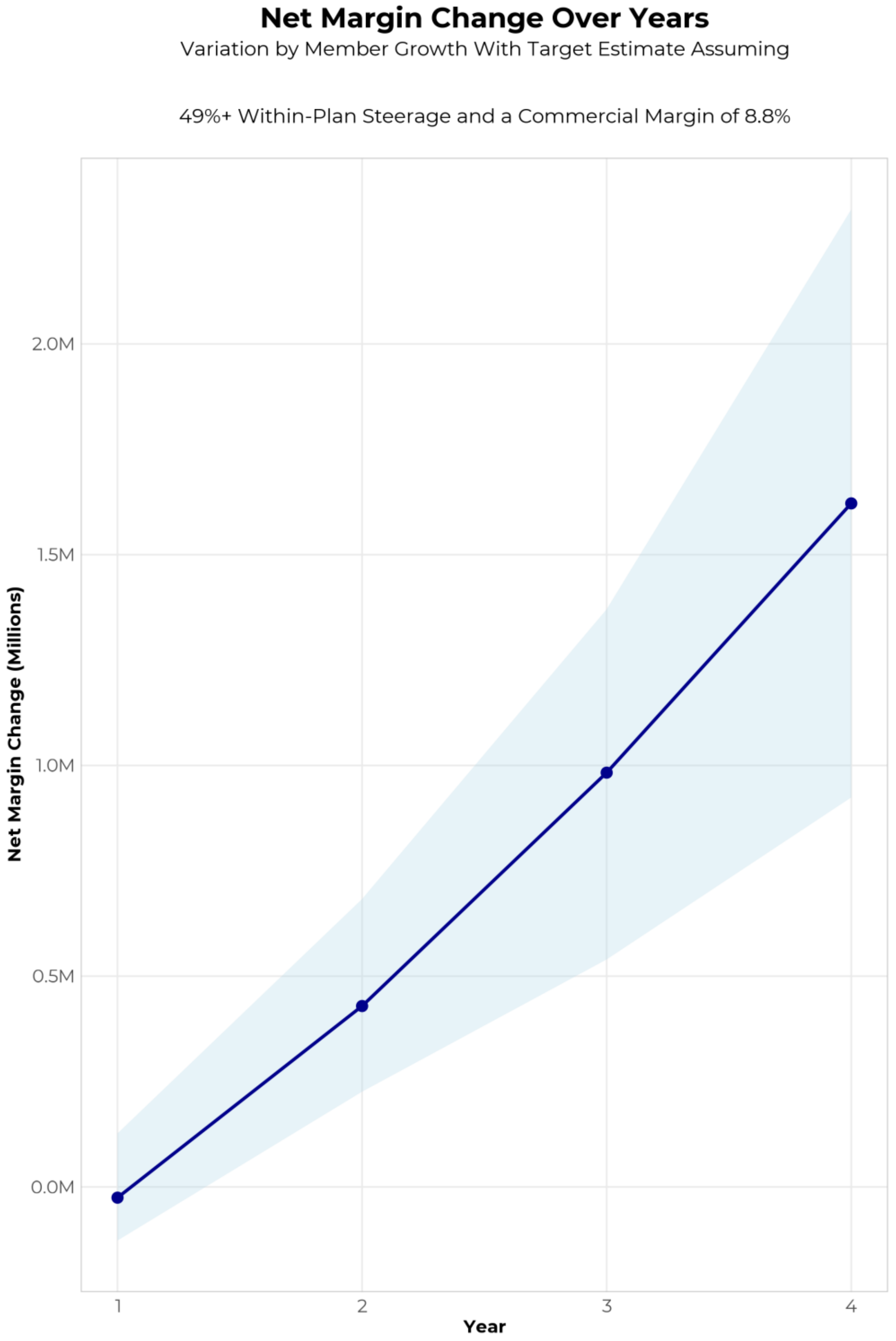
Commercial Margin Assumptions

KBA observes a Wooster Community Hospital 11% commercial margin. Conservatively, we model the DTE to be even with commercial rates given prices 10% below market, but with a 10 percentage-point increase through friendlier billing practices and improved patient collections with the patient card.

Low	Target	High
3.8%	8.8%	13.8%

KEY BENEFIT ADMINISTRATORS

FOUR-YEAR PROJECTIONS



Target Member Growth				
Metric	**Year 1**	**Year 2**	**Year 3**	**Year 4**
Number of Members	600	1,200	2,100	3,100
Incremental Margin	297.4K	594.8K	1,137.6K	1,764.4K
Network Access Fee	7.2K	14.4K	25.2K	37.2K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	**-25.4K**	**429.2K**	**982.8K**	**1,621.6K**

High Member Growth				
Metric	**Year 1**	**Year 2**	**Year 3**	**Year 4**
Number of Members	900	1,700	2,800	4,300
Incremental Margin	446.1K	842.6K	1,516.9K	2,447.4K
Network Access Fee	10.8K	20.4K	33.6K	51.6K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	**126.9K**	**683.0K**	**1,370.5K**	**2,319.0K**

Low Member Growth				
Metric	**Year 1**	**Year 2**	**Year 3**	**Year 4**
Number of Members	400	800	1,300	1,900
Incremental Margin	198.3K	396.5K	704.3K	1,081.4K
Network Access Fee	4.8K	9.6K	15.6K	22.8K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	**-126.9K**	**226.1K**	**539.9K**	**924.2K**

THE BENEFIT STEERING THE PATIENT TO WOOSTER COMMUNITY HOSPITAL

What Will this Claim Cost Me?

Wooster Community Hospital patients can have all out-of-pocket liability reduced to $0!

KEY BENEFIT ADMINISTRATORS

Explanation of Benefits	
Heart Attack, Cardiac Surgery	
THE CHARGES	
Medical Bills	$25,000
Healthcare out-of-pocket due to provider	$4,500
Since your out of pocket maximum has been met, the rest of your eligible charges this year will be paid at 100%	
THE CREDITS	
OP Radiology/X-Ray	$200
OP Diagnostic Labs	$100
Preferred Hospital Stay ($500/day, 5 days)	$2,500
Critical Illness Benefit	$2,000
Health Screening Benefit	$125
Benefit Total	$4,925
Out of Pocket Paid to Hospital	($4,500)
Balance Paid to HGA	**$425**

PROJECTED PLAN GROWTH

Year	Employees	Members	Percent of Members Choosing Wooster Community Hospital for Inpatient Care	Percent of Members Choosing Wooster Community Hospital for Outpatient Care
1	300	500	60%	50%
2	700	1,300	60%	50%
3	1,100	2,000	65%	55%
4	1,700	3,100	67%	58%
5	2,400	4,300	68%	60%
6	3,300	5,900	68%	60%
7	4,400	7,900	69%	61%
8	5,700	10,300	69%	62%
9	7,100	12,800	69%	62%
10	8,700	15,700	70%	63%
11	10,300	18,500	70%	63%
12	11,800	21,200	70%	64%
13	13,300	23,900	71%	64%
14	14,600	26,300	71%	65%
15	15,600	28,100	71%	66%
16	16,500	29,700	72%	66%
17	17,300	31,100	72%	66%
18	17,800	32,000	72%	67%
19	18,200	32,800	72%	67%
20	18,500	33,300	72%	67%

KBA currently estimates that Wooster Community Hospital has 2% inpatient market share, and 2% outpatient market compared to hospitals/clinics in the Wooster Community Hospital service area (see slide 8).

Based on previous DTE experience, KBA expects a significant bump in patient steerage to Wooster Community Hospital.

Additional steerage will occur from additional steerage patient benefits and the patient convenience card.

High estimates start nearly 10% higher than target with low estimates at 10% lower than target

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 3,100 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	$ 199,676.4	$ 345,636	$ 161,409.1	$ 706,721.5
Net Income‡	$ 17,571.52	$ 30,415.97	$ 14,204	$ 62,191.49

With DTE	Inpatient	Outpatient	Professional	Total
Revenue⁰	$ 6,158,301	$ 9,663,034	$ 4,512,555	$ 20,333,890
Net Income*	$ 541,930.5	$ 850,347	$ 397,104.8	$ 1,789,382

Total Increase in Net Income	$ 1,727,191
Network Access Fee (Revenue) of $1 PMPM	$ 37,200
Total Profit Growth	$ 1,764,391
Minus Implementation Fee (One-Time)	-$ 0
Minus Sales and Marketing (Annual – Ongoing)	-$ 180,000
Total	**$ 1,584,391**

† Assuming proportional revenues from market shares of 2% (inpatient) and 2% (outpatient)

‡ Conservatively assuming a commercial margin of 8.8% (80% of the estimated commercial margin)

⁰ Assuming 67% inpatient and 58% market steerage based on KBA narrow network DTE experience (see previous slide)

* Conservatively assuming a commercial margin of 8.8%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member convenience card.

KEY BENEFIT ADMINISTRATORS

A PARTNER YOU CAN TRUST

OUR OBJECTIVES ARE ALIGNED

WE KNOW THE MARKET

COMPLETELY TRANSPARENT

WE HAVE THE SOPHISTICATION AND INFRASTRUCTURE TO EFFECTIVELY COMPETE

KEY BENEFIT ADMINISTRATORS

Derek Moore, Senior Vice President – KBA
845-283-9973 * derek.moore@keybenefit.com